

RIVOPAM TABLET

DESCRIPTION:

RIVOPAM TABLET 0.5 MG : A white 7 mm round tablet with marking 'd' and a 'flower'.
 RIVOPAM TABLET 2MG : A white 7 mm round tablet with marking 'DUO 861' on the scored side and 'd' on another side.

COMPOSITION:

RIVOPAM TABLET 0.5 MG : Each tablet contains Clonazepam 0.5 mg.
 RIVOPAM TABLET 2 MG : Each tablet contains Clonazepam 2 mg.

PHARMACODYNAMICS:

The active substance of clonazepam is a benzodiazepine derivative, closely related to nitrazepam and diazepam, exhibiting marked anticonvulsant properties in animals. Clonazepam prevents generalisation of convulsive activity. In the majority of cases, clonazepam improves both focal seizures and primarily generalised attacks.

PHARMACOKINETICS:

Clonazepam is nearly completely absorbed after oral administration with peak serum levels being reached between 2 and 3 hours. The volume of distribution is 2-3 litres/kg. Clonazepam is metabolised by the liver and pathways include hydroxylation and reduction of the nitro group.

INDICATION:

The majority of clinical forms of epileptic disease in infants and children, especially typical or atypical petit mal epilepsies, tonic-clonic seizures generalised from the onset or secondarily, status epileptics in all its clinical forms. Clonazepam has also demonstrated efficacy in both major and minor seizures in adults including grand mal, petit mal, (classical and atypical forms) and in psychomotor, myoclonic, focal and tonic and atonic seizures.

RECOMMENDED DOSAGE:

Oral Treatment: Dosage of clonazepam is essentially individual and depends in the first instance on the age of the patient. It will be determined in each patient according to clinical response and tolerance. In order to minimise initial adverse reaction it is essential to commence with low doses and increase the daily dose progressively until a maintenance dose suited to the individual patient has been reached. Some degree of tolerance may be observed to both the adverse and therapeutic effects.

Infants	0.3 mg/day
Children 2-5 years	0.5mg/day
6-12 years	0.75 mg/day
Adults	1mg/day

The above to be in 3 or 4 divided doses with a larger dose in the evening. For children, initial dosage is based on 0.01 mg - 0.03 mg/kg per day. Average dosage range for maintenance therapy (individuals may require and tolerate larger doses)

Age	Daily dose		
	Dose in mg	Tablets 0.5 mg	Tablets 2 mg
Infants	0.5 - 1	1 - 2	¼- ½
Children	2-5 years	1.5 - 3	¾- 1½
	6-12 years	3 - 6	1½- 3
Adults	4 - 6	8 - 16	2 - 4

The daily quota should if possible be divided into three to four doses spread over the day. The maintenance dose should be attained after 2-4 weeks of treatment. To obtain optimum adjustment of the dose in infants and children, the tablets 0.5 mg is recommended. The double-scored tablets 0.5 mg facilitate administration of low doses in the early phase of treatment.

ROUTE OF ADMINISTRATION:

Oral

CONTRAINDICATIONS:

Respiratory depression and acute pulmonary insufficiency. Rivopam should not be used in patients with a history of sensitivity to the benzodiazepines.

WARNINGS & PRECAUTIONS:

Patients should abstain from alcohol since alcohol can provoke epileptic seizures, modify its action, and gives rise to unpredictable side effects. Patient's reactions like driving ability, behaviour in traffic may be modified depending on dosage and individual susceptibility. Care to be taken before administration with other CNS acting drugs or other anti epileptic drugs. Renal impairment may necessitate lower dose of clonazepam. Withdrawal of clonazepam must be done gradually. Potential for an increase in risk of suicidal thoughts or behaviours.

Risks from Concomitant Use with Opioids

Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Rivopam Tablet with opioids. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Rivopam Tablet is used with opioids. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the opioid have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of opioids (See Drug Interactions).

INTERACTION WITH OTHER MEDICAMENTS:

Opioids

Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABA_A sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see Warnings and Precautions).

Limit dosage and duration of concomitant use of benzodiazepine and opioids, and follow patients closely for respiratory depression and sedation.

USE IN PREGNANCY & LACTATION:

Use in pregnancy: It is likely that clonazepam is transferred across the placenta and clonazepam should be given to women who are or may become pregnant only when the potential benefits have been weighed against the possible hazard to the mother and child.

Studies in female rabbits doses at 2.5 mg/kg daily have shown a decrease in foetal weight and an increase in prenatal mortality.

Use in lactation: If clonazepam is given to the nursing mothers, it should be borne in mind that benzodiazepine (like clonazepam) have been found to be excreted in breast milk.

SIDE EFFECTS:

Adverse reactions to clonazepam occur in about 50% of patients and are usually referable to its sedative and muscle-relaxant effects, being most frequently somnolence 18%, fatigue 8%, muscle weakness 5%, co-ordination disturbances (ataxia) 5% and vertigo 5%. These effects occur at a higher incidence early in treatment and to a certain extent can be minimised by progressive increases in dosage to the optimum level. Clonazepam may give rise to salivary or bronchial hyper-secretion infants and small children (see *Precautions*). The effect of clonazepam on behavioural disturbances in epileptic patients is generally favourable. In 1.5% of cases, however, aggressiveness, irritability or agitation may arise during treatment. Other adverse effects reported include confusion, dysarthria, hypotension, gastric disturbances and depression. Clonazepam has provoked a generalised seizure or activated preexisting seizures in 0.5% of cases and the concurrent administration of other antiepileptics may be indicated. Rarely, the following have been reported - abnormal liver function tests, leucopenia, thrombocytopenia, eosinophilia. Sedation or drowsiness, hyperkinesia (principally in children), abnormal eye movements, coma, diplopia, dysdiadochokinesis, glassy-eyed appearance, headache, hemiparesis, nystagmus, respiratory depression, slurred speech, tremor, dizziness, vertigo, mental depression, forgetfulness, hallucinations, hysteria, increased libido, insomnia, psychosis or suicidal tendencies, muscle weakness. Increased salivation hypersecretion in upper respiratory passages, chest congestion, rhinorrhoea, shortness of breath, difficulty in swallowing occurred in infants receiving the drug, constipation, diarrhoea, encopresis, gastritis, increased or decreased appetite, weight gain or loss, dyspepsia, nausea, coated tongue, dry mouth, abnormal thirst, sore gums. Dysuria, enuresis, nocturia, urinary retention, hair loss, hirsutism, skin rash, ankle and facial oedema.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdose: Four overdose incidents have been reported to date, none was fatal. Two examples are:

1. A child, 2½ years old, who ingested 60 mg of clonazepam. Gastric lavage was performed approximately one hour later to recover a small quantity of tablets. No other treatment was undertaken. The child become drowsy and ataxic but was obviously recovering in 12 hours and was ataxic for a total of 60 hours.
2. A five year old who ingested 80 mg was stomach pumped at two hours - no result, become somnolent for 2-3 hours and then was quite recovered.

STORAGE CONDITIONS:

Store below 30°C. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE:

Please refer to outer package.

Rivopam Tablet 2mg: Consume within 2 months after opening the pouch.

PACK SIZE:

Rivopam Tablet 0.5mg: Pack in 50 x 10's in blister pack.

Rivopam Tablet 2mg: Blister pack of 100 tablets and 500 tablets

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.